<u>Package Insert</u>

GOOFICE®

1. Name of the medicinal product

1.1 Product name

GOOFICE® 5mg Film Coated Tablet

2. Quality and quantitative composition

2.1 Qualitative declaration

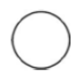
elobixibat (INN)

2.2 Quantitative declaration

Each film-coated tablet contains 5.13 mg of elobixibat hydrate (5 mg as elobixibat)

3. Pharmaceutical form

GOOFICE® is a light-yellow round-shaped film-coated tablet.

Brand name	Identifica-tion code	Appearance			Size Weight
		Face	Reverse	Lateral	
GOOFICE®	EA1				Diameter approx. 6.1 mm Thickness approx. 3.9 mm Weight approx. 110.3 mg

4. Clinical particulars

4.1 Therapeutic indications

Chronic constipation (except for constipation associated with organic diseases)

4.2 Posology and method of administration

The usual adult dose for oral use is 10 mg once daily as elobixibat before meal. The dosage may be adjusted depending on the patient’s symptoms but must not exceed the highest dose of 15 mg per day.

4.3 Contraindications

(1) Patients with medical history of hypersensitivity to the ingredients of GOOFICE®.

(2) Patients with a documented intestinal obstruction associated with a tumor or hernia or with the suspicion of such conditions. [Intestinal obstruction may be aggravated.]

4.4 Special warning and precautions for use

4.4.1. Precaution concerning indication

No clinical experience of use in drug-induced and disease-induced constipations.

4.4.2. Precaution concerning dosage and administration

GOOFICE® may cause abdominal pain or diarrhoea; dose reduction, drug interruption, or discontinuation should be considered depending on the patient's symptoms, and the need for continuing treatment with GOOFICE® should be carefully evaluated on a regular basis to avoid continuing aimless administration.

4.4.3. Careful administration (GOOFICE® should be administered with care in the following patients.)

Patient with serious liver disorder. [GOOFICE® may fail to achieve its expected efficacy in patients with biliary obstruction or reduced bile acid secretion, etc.]

4.4.4. Precaution concerning use

Precaution concerning the dispensing of the drug: Patients who are given drugs supplied in PTP package must be instructed to remove the drugs from the PTP sheet before taking drugs. [It has been reported that, if the PTP sheet is swallowed, the sharp corners of the sheet may puncture the esophageal mucosa causing perforation and resulting in serious complications, such as mediastinitis.]

4.4.5. Use in the elderly

Since the elderly generally have reduced physiological functions, cautions should be exercised, such as reducing the dose.

4.4.6. Pediatric use

Safety has not been established in low-birth-weight infants, neonates, nursing infants, infants, or pediatric patients (no clinical experience).

4.5 Interactions with other medical products and other forms of interactions

GOOFICE® exerts its inhibitory effect on P-glycoprotein. (See section 5.2 Pharmacokinetic properties).

Precautions for Coadministration (GOOFICE® should be administered with care when coadministered with the following drugs.)

Drugs	Signs, Symptoms and Treatment	Mechanism and Risk Factors
Bile acid preparations Ursodeoxycholic acid, chenodeoxycholic acid	The effects of these drugs may be attenuated.	The inhibitory effect of GOOFICE® on ileal bile acid transporter (IBAT) may interfere with reabsorption of bile acid preparations.
Aluminum-containing antacids Sucralfate hydrate, aldioxa, etc.	These drugs may attenuate the effect of GOOFICE®.	These drugs absorb bile acids in the gastrointestinal tract and may attenuate the effect of GOOFICE®.
Cholestyramine, colestimide	These drugs may attenuate the effect of GOOFICE®.	These drugs absorb bile acids and may attenuate the effect of GOOFICE®.
Digoxin, dabigatran etexilate methanesulfonate	The blood levels of these drugs may elevate and possibly enhance their effects.	Because of the inhibitory effect of GOOFICE® on P-glycoprotein (See section 5.2 Pharmacokinetic properties).
Midazolam	The blood level of midazolam may decrease, and the effect of midazolam may decrease (See section 5.2 Pharmacokinetic properties).	The mechanism is unknown.

4.6 Pregnancy, delivery or lactation

(1) GOOFICE® should be used in pregnant women and women who may possibly be pregnant only if the expected therapeutic benefits outweigh the possible risks associated with treatment. [The influences of a high dose oral administration of drug in animal studies (in rats) were observed in

maternal toxicity (1000 mg/kg/day) and survival, growth and development of offspring (350 mg/kg/day and higher).]
(2) It is advised that lactating women should avoid GOOFICE®. If treatment with GOOFICE® is essential, breast feeding must be discontinued during treatment. [In an animal experiment (in rats) using ^{14}C-elobixibat, transfer of radioactivity into milk has been reported.]

4.7 Effects on ability to drive and use machines

There is no evidence of drug effects on the ability to drive or operate machinery.

4.8 Undesirable effects

Any of the adverse reactions listed below may occur. Therefore, patients receiving GOOFICE® treatment should be carefully monitored, and if any abnormalities are noted, appropriate measures should be taken including discontinuation of GOOFICE® treatment.

Other adverse reactions

	≥5%	1 to <5%	<1%	Frequency unknown
Hepatic		Hepatic function abnormal (ALT increased, AST increased, γ-GTP increased, Al-P increased, LAP increased)		LDH increased
Central and peripheral nervous system		Dizziness		Headache
Cardiovascular				Hot flush
Gastrointestinal	Abdominal pain (23.2%), diarrhoea (14.4%)	Abdominal pain lower, abdominal distension, nausea, abdominal pain upper, abdominal discomfort, faeces soft	Stomatitis, thirst	Flatulence, defaecation urgency, vomiting, gastrointestinal sounds abnormal, constipation, colitis ischaemic, haematochezia, frequent bowel movements, faeces discoloured, anal incontinence, decreased appetite
Hypersensitivity			Urticaria	Rash
Hematologic		Anaemia	Vitamin E increased	Eosinophil count increased
Others		CK increased		Dysmenorrhoea

4.9 Overdosage

There is no data on overdose of the drug, do not exceed the dosing indicated of the drug. Actively monitor for timely response.

5. Pharmacological properties

5.1 Pharmacodynamic properties

ATC code: A06AX09

5.1.1. Mechanism of action

Elobixibat inhibits bile acid reabsorption via ileal bile acid transporter (IBAT) expressed on the epithelial cells of the terminal ileum and thereby increases the amount of bile acid passing into the large intestinal lumen. Bile acid promotes the secretion of water and electrolytes into the large intestinal lumen and enhances the colonic motility. Therefore, GOOFICE® induces the therapeutic effect on constipation.

5.1.2. Effect on constipation induced by loperamide in rats

In rats of loperamide-induced constipation model, a single oral administration of elobixibat demonstrated the effect of improving constipation.

5.2 Pharmacokinetic properties

5.2.1. Absorption

(1) A single oral dose of GOOFICE® 5 mg, 10 mg or 15 mg was administered to patients with chronic constipation before breakfast and the pharmacokinetic parameters were noted as below.

Dose	5 mg	10 mg	15 mg
Number of patients	10	10	10
C_{max} (pg/mL)	186.8±87.1	386.4±215.4	389.7±103.6
$AUC_{0-\infty}$ (pg•h/mL)	837.8±572.9	1272.5±656.2	1632.2±475.8
T_{max} (h)	1.8±1.6	1.9±1.6	1.8±0.6
$t_{1/2}$ (h)	3.3±3.1	2.5±1.5	3.2±1.5

Mean ± S.D.

(2) A single oral dose of ^{14}C-elobixibat 5 mg (approx. 2.75 MBq) was administered to healthy adult male subjects (n = 6) before breakfast and the pharmacokinetic parameters were noted as below.

Parameter	5 mg ^{14}C-elobixibat
C_{max} (nmol/L)	0.5±0.3
$AUC_{0-\infty}$ (nmol•h/L)	1.2±0.4 (n=3)
T_{max} (h)*	0.8 (0.5–2.0)
$t_{1/2}$ (h)	0.8±0.2 (n=3)

Mean ± S.D.
* Median (range)

5.2.2. Distribution

In vitro human plasma protein binding rate of elobixibat was in excess of 99% with human blood to plasma concentration ratio less than 5%.

5.2.3. Metabolism

No metabolites were observed in plasma of healthy adult male subjects (n = 6) following a single oral dose of ^{14}C-elobixibat 5 mg (approx. 2.75 MBq). Unchanged and monohydroxy forms of elobixibat were found in feces pooled over 24 to 48 hours post-dose, while the percentages of radioactivity were 96.06% and 3.16%, respectively, indicating that the majority was unchanged form.

5.2.4. Excretion

(1) When a single oral dose of GOOFICE® was administered to patients with chronic constipation under fasting conditions, the cumulative urine drug excretion rate up to 144 hours post-dose was approximately 0.01% of the amount of dose, indicating that drug excretion into urine was almost absent.
(2) When a single oral dose of ^{14}C-elobixibat 5 mg (approx. 2.75 MBq) was administered to healthy adult male subjects (n = 6), 103.1% of radioactivity dosed was excreted in feces while 0.00 to 0.02% excreted in urine up to 144 hours post-dose.

5.2.5. Drug-drug interactions

(1) IC_{50} of elobixibat towards digoxin (P-glycoprotein substrate) transport was 2.65 μmol/L in Caco-2 cells, indicating the inhibitory effect of elobixibat on P-glycoprotein.

(2) In healthy adult male and female subjects (n = 25), GOOFICE® 10 mg was orally administered once daily for 5 days with coadministration of both dabigatran etexilate 150 mg/dose/day on Day 1 and midazolam 2 mg/dose/day on Day 1 and Day 5 to compare with monoadministration of each drug. The results showed that $AUC_{0\text{-}t}$ and C_{max} of dabigatran (P-glycoprotein substrate) were 1.17 fold greater (90% confidence interval: 1.00-1.36) and 1.13 fold greater (90% confidence interval: 0.96-1.33), respectively, compared with those under monoadministration and both the upper limit of 90% confidence intervals were above 1.25 as the reference value. $AUC_{0\text{-}t}$ and C_{max} of midazolam on Day5 were 0.78-fold greater (90% confidence interval: 0.73-0.83) and 0.94-fold greater (90% confidence interval: 0.87-1.01), respectively, compared with those under monoadministration and the lower limit of 90% confidence intervals of $AUC_{0\text{-}t}$ was below 0.80 as the reference value.

5.2.6. Food effects

In patients with chronic constipation (n = 60), the effect of food intake on pharmacokinetics was evaluated following a single oral dose of GOOFICE® in a crossover design. C_{max} and $AUC_{0\text{-}\infty}$ under fed condition were approximately 20 to 30% of those under fasting one.

5.3 Preclinical safety data

Serious toxicity or toxicity limiting the use of elobixibat to the intended indication was not observed in toxicological studies performed.

5.3.1. Single-dose toxicity

In single-dose toxicity studies in mice, rats, and dogs, elobixibat was well tolerated, and the approximate lethal dose was determined to be > 2000 mg/kg in mice and rats and > 140 mg/kg in dogs.

5.3.2. Repeat-dose toxicity

In a 13-week repeat-dose oral toxicity study in mice, there were treatment-related findings associated with the liver at a very high dose. Since these findings were observed at very high exposures compared with clinical exposure, there seems to be no concern about safety in clinical practice. The frequent vomiting observed in the 4- and 13-week oral toxicity studies in dogs was considered to be a toxic change. However, there was a reduction in the number of vomiting in the 52-week oral toxicity study, and there were no findings suggesting deterioration in general conditions. Thus, vomiting was not considered seriously toxic. A wide safety margin is supported by no observed adverse effect level (NOAEL), and there seems to be no concern about safety in clinical practice. All other findings are considered to be changes related to pharmacological effects of elobixibat, changes of no toxicological importance, or secondary effects associated with administration of elobixibat.

5.3.3. Genotoxicity and carcinogenicity

Elobixibat was not genotoxic or carcinogenic.

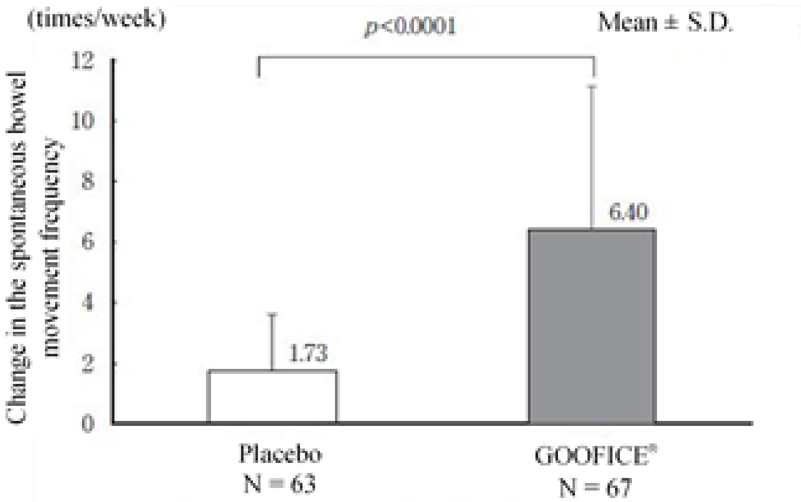
5.3.4. Reproductive and developmental toxicity

Evaluation of reproductive and developmental toxicity showed that elobixibat had no effect on fertility or embryo-fetal development. In the study for effects on pre- and postnatal development including maternal function, maternal toxicity and effects on survivability, growth, or development were observed in F1 pups at very high exposure. However, a wide safety margin is supported by NOAEL, and there seems to be no concern about safety in clinical practice.

6. Clinical Studies

6.1 Phase III Double-blind, Placebo-controlled Comparative Study

In patients with chronic constipation (n = 132), placebo or GOOFICE® 10 mg was orally administered once daily before breakfast. The change from baseline in the spontaneous bowel movement frequency on treatment period Week 1 with GOOFICE® was significantly greater than that with placebo, confirming the superiority of GOOFICE® to the placebo ($p < 0.0001$).

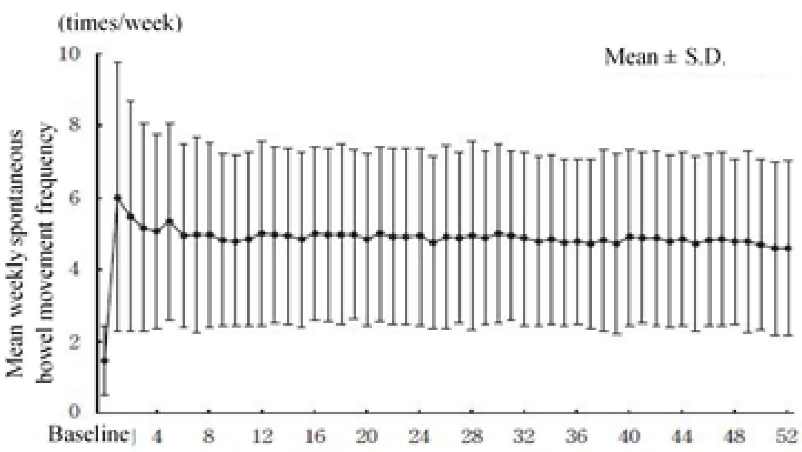


Analysis of covariance was conducted with the spontaneous bowel movement frequency of baseline as a covariate

The frequency of adverse reactions was 30% (21/69 patients). Major adverse reactions included abdominal pain 19% (13/69 patients) and diarrhoea 13% (9/69 patients).

6.2 Long-term Treatment Study

In patients with chronic constipation (n = 340), GOOFICE® 10 mg was orally administered once daily (adjusted in a range of 5 mg to 15 mg depending on the patient's symptoms) before breakfast for 52 weeks. The mean weekly spontaneous bowel movement frequency increased from baseline on treatment period Week 1, and maintained the similar level until Week 52.



The frequency of adverse reactions was 48% (163/340 patients). Major adverse reactions included abdominal pain 24% (82/340 patients), diarrhoea 15% (50/340 patients), abdominal pain lower 5% (17/340 patients), abdominal distension 3% (11/340 patients), nausea 3% (10/340 patients), liver function test abnormal 3% (10/340 patients), abdominal discomfort 2% (7/340 patients).

7. Pharmaceutical particulars

7.1 List of excipients

Microcrystalline Cellulose, D-mannitol, Hypromellose, Croscarmellose Sodium, Light Anhydrous Silicic Acid, Magnesium Stearate, Macrogol 6000, Titanium Oxide, Yellow Ferric Oxide, and Carnauba Wax

7.2 Incompatibilities

Because there are no studies on drug compatibility, do not mix this drug with other drugs.

7.3 Shelf life

GOOFICE® should be used before the expiration date indicated on the product carton.

7.4 Storage conditions

Do not store above 30ºC. (Store protected from high temperature and moisture after opening the aluminum pouch.)

7.5 Packaging

Blister pack of 10 x 10 tablets in an aluminium pack in a carton box

8. Manufacturer

EA Pharma Co., Ltd.
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9. Product Registration Holder

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10. Date of revision of the text

January 2026